جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ناهد ابراهيم عبدالظاهر مصطفى
المحافظة : القليوبيه **الحي / المدينة :** شبرا الخيمه ثان **العنوان :** //
رقم البطاقة : 26912171400764 **رقم التليفون :** 01101769506 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 17-12-1969 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط اول تاموكسيفين [14160] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ناهد ابراهيم عبدالظاهر مصطفى
National ID: 26912171400764

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

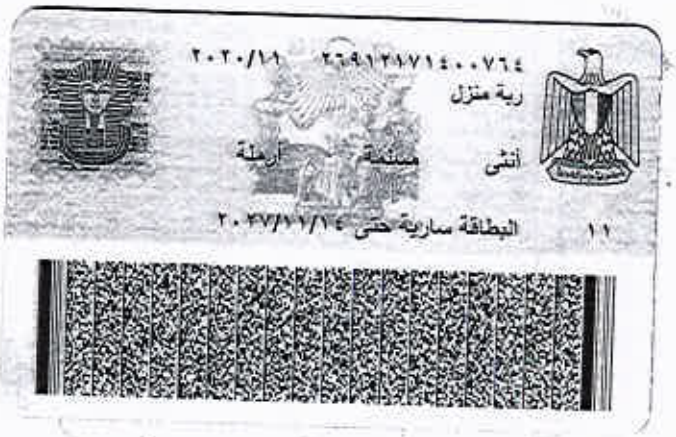
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Tamoxifen therapy**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

PATHOLOGY UNIT

Age / Gender	54 Years - Female	Visit No.	0-21903
Registered Date	2023-11-06 01:48 ص	Patient Name	[illegible]
Reporting Date	2023-11-15 11:59 ص		

PATHOLOGY REPORT

Specimen : 1. Left sided breast mass; core biopsy.
2. Left axillary lymph nodes, core biopsy.

Gross :

I. Three tan tissue cores, measured 1.1, 1.2 and 1.5cm, in length T.E. in block no. (1).
II. A fragmented (three) tissue cores, measured 0.1, 0.2 and 0.4cm, in length T.E. in block no. (2).

Microscopic :

I. Examination of serial sections reveals fibrofatty tissue cores infiltrated by malignant neoplasm arranged as sheets, nests and cords. Malignant cells exhibit moderate degree of anaplasia with moderate nuclear pleomorphism and scattered mitotic figures, tubular configuration is seen. Ductal carcinoma in situ is seen (comedo and solid).
II. Examination of serial sections reveals cores of lymphoid tissue infiltrated by same malignant infiltrate described in specimen (I).

Diagnosis :

I. Tru-cut biopsy from left breast mass, 9 o'clock:
Invasive moderately differentiated, grade 2 breast carcinoma, NST (ductal) with insitu component (solid and comedo).
II. Tru-cut biopsy from left axillary lymph nodes:
Lymphoid tissue with metastatic invasive breast (ductal) carcinoma, grade 2.

Salsabil Mahmoud

Signature
Prof. Dr. Wessam Osman
Dr. Rana Nasser
14/11/2023

Printed By : Mahmoud Hafez

20-11-2023 10:42:33

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: ناهد ابراهيم عبد الظاهر مصطفي **MR. :** 3565	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient underwent left breast cancer surgery with breast-conserving surgery and sentinel lymph node dissection following neoadjuvant chemotherapy consisting of 4 cycles of AC (doxorubicin and cyclophosphamide) and 4 cycles of THP (docetaxel, carboplatin, and trastuzumab). Post-operative pathology revealed a 3/8 estrogen receptor positive, progesterone receptor positive, HER2 3+ invasive ductal carcinoma with N1 disease. The patient received a 15-cycle adjuvant trastuzumab-based regimen (PORT + HP) and has been on tamoxifen since February 2025, with supportive therapy including dexamethasone for three months.

The patient's most recent oncology note indicates that the patient is 56 years old, has no comorbidities, and has a family history of breast cancer. The patient's left breast cancer was staged as T2 N1, ER 3/8, PR 3/8, HER2 3+. The patient received neoadjuvant chemotherapy with AC*4 and THP*4, followed by left breast conserving surgery and sentinel lymph node dissection. Post-operative pathology was consistent with a complete pathological response. The patient then received a 15-cycle adjuvant trastuzumab-based regimen (PORT + HP). The patient has been on tamoxifen since February 2025 and is currently receiving supportive therapy with dexamethasone for three months.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

PATHOLOGY UNIT

Visit No. 0-2193

Age / Gender 54 Years - Female

Patient Name ناهد ابراهيم عبد الظاهر مصطفى

Registered Date 2023-12-04 01:40 م

Reporting Date 2023-12-04 01:51 م

PATHOLOGY REPORT

Specimen :

Technique :

Immunostaining effectuated on 4 um thick paraffin sections from the paraffin block, processed in Benchmark GX (Ventana) and stained by primary antibodies against DAB as chromogen and hematoxylin as counter stain The sections were treated against: ER, PR, Her2-neu & Ki67.

Result :

ER: Faint positive nuclear staining in 2% of tumor cells (Allred score 3/8).
PR: Faint positive nuclear staining of 2% of tumor cells (Allred score 3/8).
Her2-neu: Showed positive strong complete membranous staining of >10% tumor cells (score 3+).
Ki67: Show positive staining in 35% of tumor cells.

Signature
Dr. Mariam El Rashedy.
3/12/2023.

esraa essam

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
إدارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Total DLP (mGy.cm):

Patient Name: Nahed Ibrahim Abd Al Zaher Moustafa	Date : 14/05/2024
Exam : CT urinary tract	Age : 54 Year(s)
Medical ID: 1227/2024	Pat. ID: 53556

NON-ENHANCED MULTI-SLICE CT SCAN OF THE URINARY TRACT:

Findings:

- Right double J stent is seen with its proximal end at the renal pelvis and the distal end within the urinary bladder
- No radio-dense calculi or pathological calcification seen within both kidneys, ureters or the urinary bladder.
- Normal size and parenchymal thickness of both kidneys with no back-pressure changes.
- Smooth filling of urinary bladder with mild diffuse circumferential regular mural thickening, stones or mass lesions within.
- Left ovarian well defined cystic lesion is noted measuring about 2.9 x 2.4 cm
- No other gross abnormality

Opinion:

- **Right DJ stent is seen with no right renal or ureteric stones.**
- **Mild form of cystitis**
- **Small Left ovarian cyst possibily functional for US correlation.**

Dr.Nancy Gamal

Signature Prof. Mona Tallah Hanafy

20/05/2024 06:28 م

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
مركز الدار الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

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- **Small Left ovarian cyst possibly functional for US correlation.**

Dr.Nancy Gamal

Signature Prof. Monira Fallah Hanafy

20/05/2024 06:28 م

CT Scan
[[DOCTYPE:CT_SCAN]]

Ain Shams University Hospitals
Radiology department

مستشفيات جامعة عين شمس
قسم الاشعة التشخيصية

Patient Name	ناهد ابراهيم عبدالظاهر مصطفى	PID	10210[illegible] 4220	D/T	04-02-2024

Tc^{99m} MDP whole body bone scan

History:

53-year-old patient , known case of left breast cancer .

Technique:

Whole body anterior and posterior blood-pool and delayed 3 hours scans were taken after IV injection of 20 mCi of Tc^{99m} MDP.

Findings:

a)Bone lesions:

- Homogenous tracer distribution through the surveyed skeleton
- There are no osseous lesions suspicious for metastatic disease.
- Degenerative changes of both knees, both shoulders and both sternoclavicular joints are noted

b)Other findings:

- Soft tissue contours are unremarkable .
- Both kidneys are visualized.

Impression:

The surveyed skeleton is free from metastatic osseous deposits.

Nada Badr, MSc.

Dr Sherine Sharara MD

Email : feedbackradiology@gmail.com

مستشفيات جامعة عين شمس - شارع رمسيس - العباسية القاهرة

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nahed Ibrahim Abd Al Zaher Moustafa	Date : 17/08/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 54 Year(s)
Medical ID: 1227/2024	Pat. ID: 53556

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast pathgologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows LIQ regional area of pleomorphic calcification covering an area of 5X7 cm with overlying metallic clips corresponding by US to area of ill-defined altered parenchyma measuring 3.3X1.4 cm.
- No right breast suspicious spiculated lesions or micro-calcifications.
- No right breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Few enlarged left axillary level I lymph nodes ,largest is global in configuration measuring 1.4X1 cm.
- Nonspecific right axillary LN
- Free left infra and supra-clavicular regions.

Opinion:

- Partial radiological response of the left breast LIQ malignant mass (BIRADS 6)
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. Youmna tarek

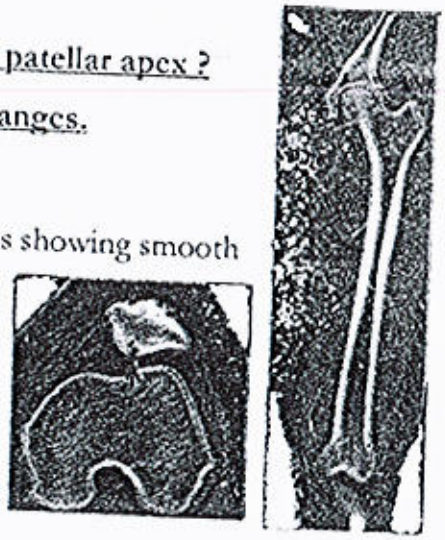
17/08/2024 11:42 AM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient ID: 767010	Patient Name: NAHID IBRAHIM ABD ELZAHIR
Study Date: 22/06/2025 07:29:52 PM	Ref. Phy.:
Age: 55Y	

Non-contrast C.T. scan of the left hip, femur and knee joint revealed:-

- Subchondral cystic changes are seen at the patellar apex ? chondromalacia patellae ? osteoarthritic changes.
- Bilateral degenerative sacroiliitis is seen.
- Normal CT appearance of the articulating bones showing smooth surface.
- No fracture seen.
- Maintained joint spaces.
- No joint effusion.
- Intact peri-articular muscles.



Dr/ Mohamed Elshazly, MD

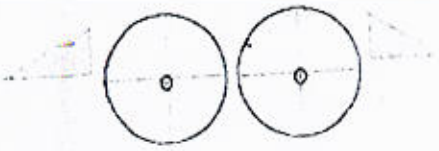
Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

3- Medical number: 83201356
Old Med :
Date 1227-2024

A-Patient ID:
1- Name : ناهد ابراهيم عبدالقادر مصطفى
Sex: Female
Patient Age : 55 Y
2-ID number:



Family history		Phone	01101769506	BMI		REASON OF MDT	
marital status		NRS Score		GROUP	C	Date of MDT	28/12/2024
Co morbidity		Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :

Mets Workup : free

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	3/8	3/8	3		

T	N	M
2	1	0

Sonomammography

PATIENT HISTORY

pt received 4AC +4TCHP--> new CR of lesion
LT BCS+SLNB: free margin
LN 0/5
complete pathological response

MDT OUTCOME & PLAN

FOR CONTINUE 1 YR HP + PORT TAM WITH LOW TOLERANCE TO STOP

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]